SACTCHECK · REGIMEN-SPECIFIC SACT CONSENT

CMF · IV · 21-day schedule

PROTOTYPE · GOVERNANCE REVIEW REQUIRED

Adjuvant when anthracyclines are unsuitable, or metastatic breast cancer · NCCP 00381 · version 6 · 21-day cycle

Patient name	DOB	Hospital no.	Date

Treatment context
After surgery: reduce recurrence risk. In metastatic disease: control cancer and symptoms. Your team confirms which setting applies.

Schedule
All three medicines are given intravenously on Day 1 of each 21-day cycle. Eight cycles for adjuvant treatment; metastatic treatment continues while beneficial and tolerated. There is no scheduled Day 8 chemotherapy dose.

1. TREATMENT DISCUSSION

- ☐ Purpose, expected benefit and alternatives explained
- ☐ Indication, medicines, sequence and duration confirmed
- ☐ Blood tests and any individually indicated heart checks discussed
- ☐ DPD result and severe fluoropyrimidine risks reviewed
- ☐ Support medicines and phase changes explained
- ☐ Fertility, contraception and breastfeeding discussed where relevant
- ☐ Common, serious and lasting drug-specific risks discussed below
- ☐ Urgent symptoms and local contact details confirmed

2. REGIMEN / AGENT-SPECIFIC MATERIAL RISKS

Cyclophosphamide
ALKYLATING CHEMOTHERAPY

- ☐ Low counts and infection
- ☐ Nausea, vomiting and hair loss
- ☐ Bladder irritation / bleeding
- ☐ Fertility effects and menopause
- ☐ Rare later blood cancers

Methotrexate
ANTIFOLATE CHEMOTHERAPY

- ☐ Low blood counts and serious infection
- ☐ Mouth ulcers and diarrhoea
- ☐ Liver injury and prolonged exposure with kidney impairment
- ☐ Uncommon serious lung inflammation

Fluorouracil (5-FU)
ANTIMETABOLITE CHEMOTHERAPY

- ☐ Low blood counts and infection
- ☐ Diarrhoea and mouth ulcers
- ☐ Rare serious heart symptoms
- ☐ Severe toxicity with DPD deficiency

Discuss the treatment setting, alternatives, exact schedule, common and serious drug-specific risks, fertility and pregnancy precautions, and the monitoring and urgent-contact plan.

Individual aim / benefit, alternatives, material risks and questions:

3. CONSENT CONFIRMATION

- ☐ Expected benefit, important risks and reasonable alternatives discussed
- ☐ Patient questions invited and answered; patient-specific material risks / priorities considered
- ☐ Emergency contact / toxicity advice and written information offered where appropriate
- ☐ Patient understands consent is voluntary and may be withdrawn

Patient / person giving consent _____ Date _____

Clinician obtaining consent _____ Date _____

Prototype only. This one-page form is a regimen-specific consent aid and requires formal governance, consultant and oncology-pharmacy review before clinical use. The current NCCP regimen, local/national consent policy, current medicine information, patient-specific material risks and clinical judgement remain authoritative.
Base regimen: NCCP 00381 v6.

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